CORTICOSTEROIDS		
Corticosteroids primarily metabolised by CYP3A (including betamethasone, budesonide, fluticasone, mometasone, prednisone, triamcinolone)	Fluticasone: in a clinical study where ritonavir 100 mg capsules twice daily were co-administered with 50 µg intranasal fluticasone propionate (4 times daily) for 7 days in healthy subjects, fluticasone propionate plasma concentrations increased significantly, whereas the intrinsic cortisol levels decreased by approximately 86% (90% CI 82-89%). Greater effects may be expected when fluticasone is inhaled. Systemic corticosteroid effects including Cushing's syndrome and adrenal suppression have been reported in patients receiving ritonavir and inhaled or intranasally administered fluticasone. The effects of high fluticasone systemic exposure on ritonavir plasma levels are unknown. Other corticosteroids: interaction not studied. Plasma concentrations of these medicinal products may be increased when co-administered with boosted PREZISTA, resulting in reduced serum cortisol concentrations.	Concomitant use of boosted PREZISTA and corticosteroids that are metabolised by CYP3A (e.g. fluticasone propionate or other inhaled or nasal corticosteroids) may increase the risk of development of systemic corticosteroid effects, including Cushing's syndrome and adrenal suppression. Co-administration with CYP3A-metabolised corticosteroids is not recommended unless the potential benefit to the patient outweighs the risk, in which case patients should be monitored for systemic corticosteroid effects. Alternative corticosteroids which are less dependent on CYP3A metabolism e.g. beclomethasone for intranasal or inhalational use should be considered, particularly for long term use.
Dexamethasone (systemic)	Not studied. Dexamethasone may decrease plasma concentrations of darunavir. (CYP3A induction)	Systemic dexamethasone should be used with caution when combined with boosted PREZISTA.
ENDOTHELIN RECEPTOR ANTAGONISTS		
Bosentan	Not studied. Concomitant use of bosentan and boosted darunavir may increase plasma concentrations of bosentan. Bosentan is expected to decrease plasma concentrations of darunavir and/or its pharmacoenhancer. (CYP3A induction)	When administered concomitantly with PREZISTA and low dose ritonavir, the patient's tolerability of bosentan should be monitored. Co administration of PREZISTA co-administered with cobicistat and bosentan is not recommended.
HEPATITIS C VIRUS (HCV) DIRECT-ACTING ANTIVIRALS		
NS3-4A protease inhibitors		
Elbasvir/grazoprevir	Boosted PREZISTA may increase the exposure to grazoprevir. (CYP3A and OATP1B inhibition)	Concomitant use of boosted PREZISTA and elbasvir/grazoprevir is contraindicated (see section 4.3).
Telaprevir 750 mg every 8 hours	telaprevir AUC ↓ 35% telaprevir C_{min} ↓ 32% telaprevir C_{max} ↓ 36% darunavir AUC_{12} ↓ 40% darunavir C_{min} ↓ 42% darunavir C_{max} ↓ 40%	It is not recommended to co-administer boosted PREZISTA and telaprevir.
Boceprevir 800 mg three times daily	boceprevir AUC ↓ 32% boceprevir C_{min} ↓ 35% boceprevir C_{max} ↓ 25% darunavir AUC ↓ 44% darunavir C_{min} ↓ 59% darunavir C_{max} ↓ 36%	It is not recommended to co-administer boosted PREZISTA and boceprevir.